



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : داليا عماد الدين عبدالهادي محمد

المحافظة : الجيزة الحي / المدينة : الهرم

رقم البطاقة : 27311122102545 رقم التليفون : 01101101973

العمر (تاريخ الميلاد) : 12-11-1973 المهنة : ربه منزل

العنوان : ..

تاريخ تسجيل الإستمارة : 28-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي
التوقيع: (.....)

المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مسح ذري علي العظام [12966] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: داليا عماد الدين عبدالهادي محمد
National ID: 27311122102545



تقرير طبي

المسيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- -- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد باليدى و تحتاج الى Bone scintigraphy

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



داليا
عماد الدين عبدالهادي محمد حسن
اش عمر بن عبدالعزيز - نزلة السويس
الهرم - الجيزة



٢٧٣١١١٢٢١٠٢٥٤٥

KJ9276777



٢٠٢١/٠٢ ٢٧٣١١١٢٢١٠٢٥٤٥

حاصله على بكالوريوس تجارة

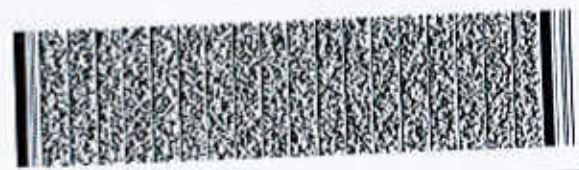
شعبة محاسبة

أنش

مطلوعة

زين العابدين احمد زكي جاد الكريم

البطاقة مارية حتى ٢٠٣١/٠٢/٠٢

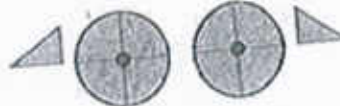


MDT FORM

Name:	Dalia Ernadein Abdelhady Mohamed	HRN:	11442
Age:	53 Y	ID:	
Menopausal Status:	post	Phone:	01101101973
NBS SCORE:		Marital Status:	
BMI:		Family History:	+ve FH
CQ Morbidity:	free	Reason Of MDT:	new

Clinical Data:

DX: Invasive duct carcinoma, Grade 3 with
ER c/s PR o/e HER o KIG: 95%



TNM

Sonomammography:

Right breast (BIRADS 5): Locally advanced suspicious process with pathological levels 1 & 2 axillary lymph nodes. Left breast (BIRADS 4c): suspicious micro-calcifications occupying the whole breast sparing the LOQ with correlate US area of altered parenchyma at UOQ. Recommendation:- Right breast : For US-guided Tru-cut biopsy and

Mets Workup:

*CT:
OPINION:
Right breast huge infiltrating the skin.
1 mass ? Right breast increased skin thickness with right breast edema. ? Right axillary enlarged LNs.
? Clear left breast and left axilla
> Few bilateral scattered tiny pulmonary nodules, about two nodules in right lung and one at left lung.
> Multiple hepatic metastasis as detailed
> Bilateral iliac bones and multilevel vertebral levels tiny lytic foci for bone scan assessment.

History:

Left breast mass, Tru cut biopsy: - Fibroadenosis, duct ectasia & periductal mastitis. No malignancy

Clinical: rt IBC with fixed axillary LNs for possibility of systemic tx
omni assay

Recommendations:

MDT Outcome & Plan

for taxol +/-carboplatin if embolizing visceral crises

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Dr. Hany Elmaghrabi					

Name: MR. :	داليا عمادالدين عبدالهادي محمد 11442	
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمناظرة وجد انه The patient's weight is 75 kg, height 160 cm, and calculated body surface area is 1.82 m²; taxol 127 mg was administered in week 1. A CT scan demonstrated a few bilateral scattered tiny pulmonary nodules measuring 2–3 mm (approximately two nodules in the right lung and one in the left lung). The liver is of average size but contains multiple bilobar hepatic focal lesions, the largest measuring 3 × 2.8 cm. AST and ALT are elevated, while total and direct bilirubin are normal, indicating no impending visceral crisis. The plan is to continue taxol as a single agent, with taxol week 2 scheduled, followed by week 3 pharmacy and week 4 clinic visits for CBC and liver enzymes. The patient has a diagnosis of right-sided breast cancer, clinical stage cT4 N2 M1, with multilobar hepatic focal lesions (HFL) and invasive ductal carcinoma grade 2, triple-negative, Ki-67 95 %. Bilateral iliac bone lesions are present but bone scan is free. Pulmonary nodules remain 2–3 mm. On examination, right breast shows redness and edema. Taxol 140 mg was administered in week 4, with plans for weeks 5 and 6; AST is 56 U/L and ALT 87 U/L, showing a decrease. For week 5, taxol 140 mg was administered; laboratory values from 27 July were reviewed and accepted. For week 6, taxol 140 mg was administered; laboratory values from 27 July were reviewed and accepted. On examination, a right-breast mass measuring 7 × 6 cm with edema and redness is noted, and liver enzymes remain elevated (ALT 73 U/L, AST 98 U/L). Given the sluggish local response and persistent hepatic enzyme elevation, carboplatin (AUC 2) was added to the regimen for week 7, with taxol 140 mg continued, along with liver support measures. For week 8, taxol 140 mg combined with carboplatin (AUC 2) was administered, accompanied by liver support and a proton-pump inhibitor (emmx) for nausea. Examination shows right-breast redness, a mass now 7 × 8 cm, and a right axillary lymph node measuring 1 × 2 cm. For week 9, taxol 140 mg plus carboplatin (AUC 2) was administered; laboratory results from 17 August were reviewed and accepted. On examination at week 10, right-breast edema and induration have decreased, and the right axillary lymph node is now 2 × 3 cm. AST is 57 U/L and ALT 62 U/L. Taxol 140 mg and carboplatin (AUC 2) were administered in week 10, with plans for weeks 11 and 12 and no significant toxicity noted. The patient has completed week 10 of the taxol 140 mg and carboplatin AUC 2 regimen.		
تحريراً في ,		
	الوقت :	التاريخ :
	dina makram	19-09-2026
الطبيب المعالج :		
يعتمد ,		

Patient's Name: DALIA EMAD EL DIN A. EL HADY
Date: 2026-04-15
Referred by: Prof.Dr.
Hospital: Goustave Roussey International Egypt

داليا محمد
الحادي
2026/04/15

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:
Clinical summary: 52-year-old lady with positive family history (Maternal side grandmother an multiple cousins) complaining of right enlarged breast.

Findings:

Right breast:

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- Thickened skin and coarsened trabeculae.
- Global asymmetry of the whole breast with widespread distortion.
- Ultrasound demonstrated infiltration of the breast with heterogenous hypoechoic solid mass lesion with surrounding distortion measuring about 12 x 8 x 10 cm reaching the nipple as well as the overlying skin.
- Thickened skin reaching 8 mm with coarsened trabeculae and subdermal & interstitial edema.
- Multiple (more than four) pathological axillary levels 1 & 2 LNs are noted, showing eccentric hila and focal cortical thickness reaching 7 mm. No pathological level 3 axillary or supra-clavicular lymph nodes.

Left breast:

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- upper breast and lower outer quadrants showing segmental distribution at the Outer quadrants & clusters at the LOQ of suspicious pleomorphic micro-calcifications measuring about 11 x 7 x 6.5 cm reaching the nipple accompanied with surrounding subtle distortion.
- Ultrasound demonstrated area of altered parenchyma seen at 12 -2 o'clock occupying the middle and posterior thirds measuring more than 6 x 1.5 cm showing prominent ducts, nodularity and multiple minute cysts. This area is seen continuous with UOQ segmental dilated ducts seen reaching the nipple. No US detected abnormality at the LOQ or UIQ.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 5): Locally advanced suspicious process with pathological levels 1 & 2 axillary lymph nodes.

Left breast (BIRADS 4c): suspicious micro-calcifications occupying the whole breast sparing the LOQ with correlate US area of altered parenchyma at UOQ.

Recommendation:-

Right breast : For US-guided Tru-cut biopsy and histopathological correlation.

Left breast: For US-guided Tru-cut biopsy of the UOQ US correlate area and histopathological correlation, if failed for stereotactic biopsy.

Best Regards,
DR. Amal Hegazi, M.Sc.

①

Patient Name	: الأستاذ / داليا عماد الدين عبد الهادي	Request No.	: 13006018
Patient No.	: 27311122102545	Request Date	: 21/04/2026
Path No.	: 1105/26	Verification Date	: 05/05/2026
Sex	: Female	Financial Category	: جومئلف رومى - مياره
Age	: 52 Y	Department	: Out Patient
Refereing Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical data: Left & right breast lesions.

Nature of specimen:

- 1) Left breast: Tru cut biopsy, collected on 21/4/2026, 2:40 pm, cold ischemia time is 20 seconds.
- 2) Right breast: TrU cut biopsy collected on 21/4/2026, at 2:50 pm, cold ischemia time is 20 seconds.

Gross examination:

- 1) Multiple fragmented grayish white tissue cores largest is 1 cm, totally submitted.
- 2) Multiple fragmented grayish white tissue cores 0.8 cm, totally submitted.

Microscopic examination:

- 1) Left breast:
 - Histopathological examination revealed a cores of breast tissue showing diffuse stromal fibrosis and focal adenosis. Some of the proliferating ducts are irregularly dilated. The ducts are lined by inner columnar and outer myoepithelial cells. No atypia or malignancy in sections examined.

2) Right breast

- * Histological type: Invasive duct carcinoma with areas of necrosis.
- * Grade (differentiation + nuclear pleomorphism + mitosis): (3+3+2) Grade 3
- * DCIS: Absent.
- * Angiolinvasion: Absent.
- * Perineural invasion: Absent.

Diagnosis

- 1) Left breast mass, Tru cut biopsy:
 - Fboradenosis, duct ectasia & periductal mastitis. No malignancy.

Verified by :

Dr.Heba Hasan

Printing Date : 05/05/2026 12:54:24

Page 1 of 2

Printing By : Ali mostafa



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / داليا عبد الدين عبد الهادي	Request No. : 13006018
Patient No. : 27311122102545	Request Date : 21/04/2026
Path No. : 1105/28	Verification Date : 05/05/2026
Sex : Female	Financial Category : جوماتف روسي - ميكره
Age : 52 Y	Department : Out Patient
Referring Doc. :	IHC No. :
	Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass diagnosed as invasive duct carcinoma grade 3.
Nature of Specimen: Tru-cut biopsy.

Technique:

- Sections were prepared from the paraffin block of right breast, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: No nuclear reaction in tumor cells (Allred score 0/8).
- PR: No nuclear reaction in tumor cells (Allred score 0/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 95 %

Conclusion:

Right breast cancer, Tru-cut biopsy:

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67: 95 %

* Scoring system: *ER, PR*

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

Verified by :

Dr.Heba Hasan

Printing Date : 05/05/2026 12:54:46

Page 1 of 2

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mistr El Qadema - Cairo, Egypt

3



Patient's Name: Dalia Imad Eldin Abdelhadi Mohammed
Date: 2026-06-09
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Clinical History:

- **New case of right breast cancer.**

Findings:

- Right breast huge mass 8x7cm infiltrating the skin.
- Right breast increased skin thickness with right breast edema.
- Right axillary enlarged LNs largest 22mm.
- Clear left breast and left axilla.
- Few bilateral scattered tiny pulmonary nodules are seen of average size 2-3 mm, about two nodules in right lung and one at left lung.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

OPINION:

- Right breast huge mass infiltrating the skin.
- Right breast increased skin thickness with right breast edema.
- Right axillary enlarged LNs.
- Clear left breast and left axilla.
- Few bilateral scattered tiny pulmonary nodules, about two nodules in right lung and one at left lung.

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER I.V. CONTRAST ADMINISTRATION: -

②



Patient's Name: Dalia Imad Eldin Abdelhadi Mohammed
Date: 2026-06-09
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Findings:

- The liver is of average size with multiple bilobar hepatic focal lesions largest is seen at left lobe measuring 3x2.8cm.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Bilateral iliac bones and multilevel vertebral levels tiny lytic foci for bone scan assessment.

Opinion:

- Multiple hepatic metastasis as detailed.
- Bilateral iliac bones and multilevel vertebral levels tiny lytic foci for bone scan assessment.

Dr. Mai Shaaban MD

Dr.Mai Shaaban

Dr.Mai Shaaban

5



SCAN 8 LABS

مركز النيل للأشعة والتحاليل

Name : Dalia Imad Eldin Abdelhadi Mohammed

Date : 15-06-2026 07:08

Branch : Dokki

Medical No. : 393975

Ref.Physician Prof : Goustav Roussey Egypt

Technetium-99m MDP Whole-Body Bone Scintigraphy Report

Indication: Staging/evaluation of right breast carcinoma in a 53-year-old female patient; complaining of bone aches.

Technique

Whole-body anterior and posterior planar osseous imaging was performed following the intravenous administration of Tc-99m MDP.

Findings

- **Axial Skeleton:** The skull, spinal column, sternum, ribs, and pelvic girdle show normal, homogeneous, and symmetrical radiotracer distribution. No localized areas of abnormally increased or decreased uptake are detected.
- **Appendicular Skeleton:** Both upper and lower extremities show expected, physiological tracer uptake throughout the long bones and articular surfaces, with no focal abnormalities.
- **Kidneys and Bladder:** Symmetrical tracer excretion is noted in both kidneys, with normal physiological visual representation of the urinary bladder.

Conclusion

- **No scintigraphic evidence of osteoblastic metastatic bone disease.**

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

6

Call Center 19656

Medical No. : 2619

Patient's Name: Dalia Imad Eldin Abdelhadi

Mohammed

Date: 2026-09-17

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS AFTER ORAL AND I.V. CONTRAST INJECTION**
REVEALED:

I- CHEST:

- No pulmonary masses or nodules
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No dilated biliary radicles. Multiple hepatic hypodense focal lesions the largest is seen in the left lobe measuring 38 x 31 mm showed heterogenous mainly peripheral enhancement
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.

Medical No. : 2619

Patient's Name: Dalia Imad Eldin Abdelhadi

Mohammed

Date: 2026-09-17

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple vertebral bodies and bilateral iliac bones sclerotic lesions

Opinion:

- ❖ Known case of right breast cancer.
- ❖ Multiple hepatic and osseous metastatic lesions.

Dr. Asser Ahmed

Medical No. : 2619

Patient's Name: Dalia Imad Eldin Abdelhadi

Mohammed

Date: 2026-09-17

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



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Medical No. : 2619

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Dr. Asser Ahmed

Medical No. : 2619

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Mohammed

Date: 2026-09-17

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



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Patient's Name: Dalia Imad Eldin Abdelhadi

Mohammed

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Hospital: Goustave Roussey International Egypt



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- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple vertebral bodies and bilateral iliac bones sclerotic lesions

Opinion:

- ❖ Known case of right breast cancer.
- ❖ Multiple hepatic and osseous metastatic lesions.

Dr. Asser Ahmed

Medical No. : 2619

Patient's Name: Dalia Imad Eldin Abdelhadi

Mohammed

Date: 2026-09-17

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



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ABDOMEN AND PELVIS AFTER ORAL AND I.V. CONTRAST INJECTION**
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Medical No. : 2619

Patient's Name: Dalia Imad Eldin Abdelhadi

Mohammed

Date: 2026-09-17

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



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- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple vertebral bodies and bilateral iliac bones sclerotic lesions

Opinion:

- ❖ Known case of right breast cancer.
- ❖ Multiple hepatic and osseous metastatic lesions.

Dr. Asser Ahmed

Morse Fall Risk Assessment Reassessment, and Interventions

الرقم الطبي:	11042
الاسم العربي:	داليا عمار الدين
اسم المريض:	عبدالهادي محمد
تاريخ الميلاد:	12/11/1973
الرقم الطبي:	المريض

Item	Scale	Date/ Score							
1. History of falling (immediate or within 3 months)	No 0 Yes 25	6/7							
2. Secondary diagnosis (≥ 2 medical diagnoses in chart) • Secondary diagnosis • Post-Surgery • Medication	No 0 Yes 15	0							
3. Ambulatory aid bedrest/ nurse assist Crutches/cane/walker Furniture	0 15 30	0							
4. Intravenous therapy/ heparin lock	No 0 Yes 20	0							
5. Gait: Normal/bedrest/wheelchair Weak Impaired*	0 10 20	0							
6. Mental status: Oriented to own ability Overestimates/ forgets limitations High risk (Bed ridden Patients all ICU, dialysis, OR,	0 15 15	0							
Score:	Initial:	0							
Nurse Name/ID									
Nurse signature									

mm/dd/yyyy

0

MDT FORM

Name:	Dalla Emadeidin Abdelhady Mohamed	MRN:	11442
Age:	53 Y	ID:	
Menopausi Status:	post	Phone:	01101101973
NRS SCORE:		Marital Status:	
BMI:		Family History:	+ve FH
CO Morbidity:	free	Reason Of MDT:	new

Clinical Data:

BX: Invasive duct carcinoma, Grade 3 with
ER 0/8 **PR** 0/8 **HER** 0 **Ki67** 95%

R

L

Sonomammography:

Right breast (BIRADS 5): Locally advanced suspicious process with pathological levels 1 & 2 axillary lymph nodes, Left breast (BIRADS 4c): suspicious micro-calcifications occupying the whole breast sparing the LOQ with correlate US area of altered parenchyma at UOQ. Recommendation:- Right breast : For US-guided Tru-cut biopsy and

Mets Workup:

*CT:

OPINION:

Right breast huge infiltrating the skin,
1 mass ? Right breast increased skin thickness with right breast edema.: ? Right axillary enlarged LNs.
? Clear left breast and left axilla
> Few bilateral scattered tiny pulmonary nodules, about two nodules in right lung and one at left lung.
> Multiple hepatic metastasis as detailed
> Bilateral iliac bones and multilevel vertebral levels tiny lytic foci for bone scan assessment.

History:

Left breast mass, Tru cut biopsy: - Fhoradenosis, duct ectasia & periductal mastitis. No malignancy clinically: rt IBC with fixed axillary LNs for possibility of systemic tt ornila essayy

Recommendations:

MDT Outcome & Plan

for taxol +/-carboplatin if embtending visceral crises

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
أ. د. عبد الله بن عبد الله					

mm/dd/yyyy

MDT FORM

Name:	Dalia Enaddidin Abdelhady Mohamed	MRN:	11442
Age:	53 Y	ID:	
Menopausal Status:	post	Phone:	01101101973
NRS SCORE:		Marital Status:	
BMI:		Family History:	+ve PH
CO Morbidity:	free	Reason Of MDI:	new

Clinical Data:

R L

BX: Invasive duct carcinoma, Grade 3 with
ER 0/8 PR 0/8 HER 0 KI67 95%

Sonomammography:

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Mets Workup:

*CT:

OPINION:

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> Bilateral iliac bones and multilevel vertebral levels tiny lytic foci for bone scan assessment.

History:

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Clinically: rt IBC with fixed axillary LNs for possibility of systemic tt omnia essay

Recommendations:

MDT Outcome & Plan

for taxol +/-carboplatin if emboding visceral crises



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. حنان عراب					

mm/dd/yyyy

MDT FORM

Name:	Dalia Emadelain Abdelhady Mohamed	MRN:	11442
Age:	53 Y	ID:	
Menopausal Status:	post	Phone:	01101101973
NRS SCORE:		Marital Status:	
BMI:		Family History:	+ve FH
CO Morbidity:	free	Reason Of MDT:	new

Clinical Data:

R L

BX: Invasive duct carcinoma, Grade 3 with
ER 0/8 PR 0/8 HER 0 Ki67 95%

Sonomammography:

Right breast (BIRADS 5): Locally advanced suspicious process with pathological levels 1 & 2 axillary lymph nodes, Left breast (BIRADS 4c): suspicious micro-calcifications occupying the whole breast sparing the LOQ with correlate US area of altered parenchyma at UOQ. Recommendation:- Right breast : For US-guided Tru-cut biopsy and

Mets Workup:

*CT:
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> Bilateral iliac bones and multilevel vertebral levels tiny lytic foci for bone scan assessment.

History:

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clinically: rt IBC with fixed axillary LNs
for possibility of systemic tt
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Recommendations:

MDT Outcome & Plan

for taxol +/-carboplatin if emboding visceral crises



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. حنان حورث					



جمهورية مصر العربية
بطاقة تعقب الشخصية

داليا
عماد الدين عبدالهادي محمد حسن
اش عشر بن عبدالعزيز-نزلة السيسى
الهرم - الجيزة



KJ9276777

٢٧٣ ١١ ١٢ ٢١ ٠٢٥ ٤٥

٢٠٢٤/٠٢ ٢٧٣١١٢٢١٠٢٥٤٥
حاصله على بكالوريوس تجاره
شعبة محاسبه
انثى مسلمة متزوجة
زين العابدين احمد زكى جادالكريم
البطاقة سارية حتى ٢٠٣١/٠٢/٠٢ ١



Patient's Name: DALIA EMAD EL DIN A. EL HADY
Date: 2026-04-15
Referred by: Prof.Dr.
Hospital: Goustave Roussey International Egypt



Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 52-year-old lady with positive family history (Maternal side grandmother an multiple cousins) complaining of right enlarged breast.

Findings:

Right breast

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- Thickened skin and coarsened trabeculae.
- Global asymmetry of the whole breast with widespread distortion.
- Ultrasound demonstrated infiltration of the breast with heterogeneous hypoechoic solid mass lesion with surrounding distortion measuring about 12 x 8 x 10 cm reaching the nipple as well as the overlying skin.
- Thickened skin reaching 8 mm with coarsened trabeculae and subdermal & interstitial edema.
- Multiple (more than four) pathological axillary levels 1 & 2 LNs are noted , showing eccentric hila and focal cortical thickness reaching 7 mm. No pathological level 3 axillary or supra-clavicular lymph nodes.

Left breast

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- upper breast and lower outer quadrants showing segmental distribution at the Outer quadrants & clusters at the LOQ of suspicious pleomorphic micro-calcifications measuring about 11 x 7 x 6.5 cm m, reaching the nipple accompanied with surrounding subtle distortion.
- Ultrasound demonstrated area of altered parenchyma seen at 12 -2 o'clock occupying the middle and posterior thirds measuring more than 6 x 1.5 cm showing prominent ducts, nodularity and multiple minute cysts. This area is seen continuous with UOQ segmental dilated ducts seen reaching the nipple. No US detected abnormality at the LOQ or UIQ.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 5): Locally advanced suspicious process with pathological levels 1 & 2 axillary lymph nodes.

Left breast (BIRADS 4c): suspicious micro-calcifications occupying the whole breast sparing the LOQ with correlate US area of altered parenchyma at UOQ.

Recommendation:-

Right breast : For US-guided Tru-cut biopsy and histopathological correlation.

Left breast: For US-guided Tru-cut biopsy of the UOQ US correlate area and histopathological correlation, if failed for stereotactic biopsy.

Best Regards,

DR. Amal Hegazi, M.Sc.

①

Patient Name : الأستاذ / داليا عماد الدين عبد الهادي

Patient No. : 27311122102545

Path No. : 1105/26

Sex : Female

Age : 52 Y

Referreing Doc :

Request No. : 13006018

Request Date : 21/04/2026

Verification Date : 05/05/2026

Financial Category : جوستاف روسي - مبادره

Department : Out Patient

Registered by : .marwa mohamed youss

Pathology Report

Clinical data: Left & right breast lesions.

Nature of specimen:

- 1) Left breast: Tru cut biopsy, collected on 21/4/2026, 2:40 pm, cold ischemia time is 20 seconds.
- 2) Right breast: TrU cut biopsy collected on 21/4/2026, at 2:50 pm, cold ischemia time is 20 seconds.

Gross examination:

- 1) Multiple fragmented grayish white tissue cores largest is 1 cm, totally submitted.
- 2) Multiple fragmented grayish white tissue cores 0.8 cm, totally submitted.

Microscopic examination:

- 1) Left breast:

- Histopathological examination revealed a cores of breast tissue showing diffuse stromal fibrosis and focal adenosis. Some of the proliferating ducts are irregularly dilated. The ducts are lined by inner columnar and outer myoepithelial cells. No atypia or malignancy in sections examined.

- 2) Right breast

- * Histological type: Invasive duct carcinoma with areas of necrosis.
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (3+3+2) Grade 3
- * DCIS: Absent.
- * Angioinvasion: Absent.
- * Perineural invasion: Absent.

Diagnosis

- 1) Left breast mass, Tru cut biopsy:
- Fboradenosis, duct ectasia & periductal mastitis. No malignancy.

Verified by :

Dr.Heba Hasan

2

Patient Name :	الأستاذة / داليا عماد الدين عبد الهادي	Request No. :	13006018
Patient No. :	27311122102545	Request Date :	21/04/2026
Path No. :	1105/26	Verification Date :	05/05/2026
Sex :	Female	Financial Category :	جورستاف روسي - مبادره
Age :	52 Y	Department :	Out Patient
Referreing Doc. :		IHC No. :	
		Registered by :	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass diagnosed as invasive duct carcinoma grade 3.
Nature of Specimen: Tru-cut biopsy.

Technique:

- Sections were prepared from the paraffin block of right breast, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: No nuclear reaction in tumor cells (Allred score 0/8).
- PR: No nuclear reaction in tumor cells (Allred score 0/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 95 %

Conclusion:

Right breast cancer, Tru-cut biopsy:

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67: 95 %

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and

Verified by :

Dr.Heba Hasan

Printing Date : 05/05/2026 12:54:46

Page 1 of 2

Printed By : Ali mostafa



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

3

Patient's Name: Dalia Imad Eldin Abdelhadi Mohammed
Date: 2026-06-09
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Clinical History:

- **New case of right breast cancer.**

Findings:

- Right breast huge mass 8x7cm infiltrating the skin.
- Right breast increased skin thickness with right breast edema.
- Right axillary enlarged LNs largest 22mm.
- Clear left breast and left axilla.
- Few bilateral scattered tiny pulmonary nodules are seen of average size 2-3 mm, about two nodules in right lung and one at left lung.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

OPINION:

- Right breast huge mass infiltrating the skin.
- Right breast increased skin thickness with right breast edema.
- Right axillary enlarged LNs.
- Clear left breast and left axilla.
- Few bilateral scattered tiny pulmonary nodules, about two nodules in right lung and one at left lung.

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER I.V. CONTRAST ADMINISTRATION: -

3

Patient's Name: Dalia Imad Eldin Abdelhadi Mohammed
Date: 2026-06-09
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Findings:

- The liver is of average size with multiple bilobar hepatic focal lesions largest is seen at left lobe measuring 3x2.8cm.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Bilateral iliac bones and multilevel vertebral levels tiny lytic foci for bone scan assessment.

Opinion:

- Multiple hepatic metastasis as detailed.
- Bilateral iliac bones and multilevel vertebral levels tiny lytic foci for bone scan assessment.

Dr. Mai Shaaban MD

Dr.Mai Shaaban

Dr.Mai Shaaban

5

Name : Dalia Imad Eldin Abdelhadi Mohammed
Branch : Dokki

Date : 15-06-2026 07:08

Medical No. : 393975

Ref.Physician Prof : Goustav Roussey Egypt

Technetium-99m MDP Whole-Body Bone Scintigraphy Report

Indication: Staging/evaluation of right breast carcinoma in a 53-year-old female patient; complaining of bone aches.

Technique

Whole-body anterior and posterior planar osseous imaging was performed following the intravenous administration of Tc-99m MDP.

Findings

- **Axial Skeleton:** The skull, spinal column, sternum, ribs, and pelvic girdle show normal, homogeneous, and symmetrical radiotracer distribution. No localized areas of abnormally increased or decreased uptake are detected.
- **Appendicular Skeleton:** Both upper and lower extremities show expected, physiological tracer uptake throughout the long bones and articular surfaces, with no focal abnormalities.
- **Kidneys and Bladder:** Symmetrical tracer excretion is noted in both kidneys, with normal physiological visual representation of the urinary bladder.

Conclusion

- No scintigraphic evidence of osteoblastic metastatic bone disease.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

6